MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY VOLUME 10 OF 28

MFT 710

Couples Therapy

16 teaching weeks 1 practical 4 worked cases 6 figures

SEMESTER TWO

1.71 units Prerequisite: MFT 702, 706

> Couple work is the largest single referral stream in most family therapy practices and the most demanding therapeutically: two people, both of whom want you to agree with them.

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 710 Couples Therapy

MSc Marriage and Family Therapy Semester Two 1.71 Units 16 teaching weeks Prerequisite: MFT 702, 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14 16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	**Couples Therapy**	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

Contents

How to use this book . . . 5

1. Course description . . . 5

2. Learning outcomes . . . 5

3. Assessment . . . 5

4. The sixteen-week plan . . . 6

Week 1 What couples want, and what therapy offers . . . 6

Week 2 Assessment I . . . 7

Week 3 Individual sessions, IPV screening and secrets . . . 7

Week 4 The negative cycle . . . 8

Week 5 Attachment applied to couples . . . 9

Week 6 EFT I: tracking and de-escalation . . . 10

Week 7 Observed couple assessment . . . 11

Week 8 EFT II: restructuring . . . 11

Week 9 Behavioural couple therapy . . . 12

Week 10 The Gottman research programme . . . 13

Week 11 Infidelity and betrayal trauma . . . 14

Week 12 Sex and intimacy . . . 15

Week 13 Violence, control and contraindications . . . 16

Week 14 Separation, divorce and stepfamilies . . . 16

Week 15 Culture, faith and family of origin . . . 16

Week 16 Consolidation and ending . . . 17

Glossary . . . 17

Self-assessment . . . 17

- Multiple choice . . . 17
- Answer key . . . 18
- Short answer . . . 18
- Essay (choose one, 2,000 words) . . . 18

Further reading . . . 18

Course leader's summary. Couple work is the largest single referral stream in most family therapy practices and the most demanding therapeutically: two people, both of whom want you to agree with them. This course integrates attachment-based (EFT), behavioural (TBCT/IBCT) and systemic approaches, and takes infidelity, violence, separation and cultural context seriously.

How to use this book

Couple work is where the therapist's own relationship history is most easily activated. The reflective tasks in this book are not optional extras; they are the difference between a clinician who can hold two perspectives at once and one who quietly takes sides. Do them.

1. Course description

The course covers: assessment of the couple (including individual sessions and violence screening), the major evidence-based models (EFT and behavioural couple therapy), the Gottman research programme, communication training, infidelity and betrayal trauma, sexual problems, separation and divorce work, and cultural and religious context.

Three frameworks are integrated: **attachment** (why the bond matters), **behaviour** (what maintains the pattern), and **system** (how the pattern organises itself).

2. Learning outcomes

#	Outcome	Assessed
LO1	Conduct a couple assessment safely, including IPV screening and individual sessions	Observed assessment
LO2	Formulate the couple's negative cycle in attachment terms	Case assignment
LO3	Deliver core EFT and behavioural interventions under supervision	Observed sessions
LO4	Work with infidelity, including disclosure, meaning and rebuilding	Case study
LO5	Address sexual concerns within the couple frame	Exam
LO6	Manage separation, divorce and stepfamily transitions ethically	Case study
LO7	Adapt couple work to cultural, religious and extended-family context	Viva

3. Assessment

Component	Weight	Due
Couple assessment observed (40 min)	25%	Week 7
Formulation and treatment plan (2,000 words)	25%	Week 10
Recorded or live session with analysis (30 min)	30%	Week 15

Component	Weight	Due
Examination	20%	Week 16

4. The sixteen-week plan

Week	Topic	Practical
1	What couples want and what therapy offers; the evidence	Map a couple's goals
2	Assessment I: structure, history, and the couple narrative	Intake role-play
3	Assessment II: individual sessions, IPV screening, secrets	Screening rehearsal
4	The negative cycle: pursue withdraw and its variants	Cycle mapping
5	Attachment theory applied to couples	Attachment interview
6	EFT I: tracking and de-escalation	Enactment drill
7	**Observed couple assessment**	Assessed
8	EFT II: restructuring and softening events	Softening rehearsal
9	Behavioural couple therapy: exchange and communication	Speaker listener
10	**Formulation and plan due**; the Gottman programme	Four horsemen practice
11	Infidelity and betrayal trauma	Disclosure planning
12	Sex and intimacy in couple therapy	Sensate focus design
13	Violence, control and when couple therapy is contraindicated	Safety planning
14	Separation, divorce, co-parenting and stepfamilies	Co-parenting mediation
15	Culture, faith and family-of-origin in couple work	Cultural adaptation
16	Consolidation, relapse prevention and ending	**Session analysis due**

Week 1 What couples want, and what therapy offers

Couples arrive with a presenting complaint (money, sex, in-laws, phone) and an underlying bond question ("are you there for me?"). The complaint is the doorway; the bond is the room.

The evidence, briefly. Couple therapy is effective for relationship distress with moderate-to-large effects. Behavioural couple therapy, cognitive-behavioural couple therapy, and emotionally focused therapy have the strongest evidence base. Emotionally focused therapy has the most consistent evidence for durability, including in high-distress couples.

What predicts a good outcome. Early change, motivation of both partners, shared definition of the problem, low coercive control, and consistently the quality of the shared therapeutic alliance.

What predicts failure. Coercive control, undisclosed affairs that surface mid-treatment, untreated addiction, severe individual psychopathology, and one partner who has already decided to leave but has not said so.

> **REFLECT**
> What did your own family teach you about conflict, repair, and what a marriage is for? Write 400 words. Keep it; you will return to it in Week 13.

Week 2 Assessment I

Structure of a couple assessment (2 3 sessions).

1. **Joint session:** both partners' accounts, the history of the relationship, the current pattern, and the goals.
2. **Individual sessions:** one with each partner risk screen, secrets policy, individual history, ambivalence, and "is there anything you would not say in front of your partner?"
3. **Feedback session:** formulation, plan, and the agreement.

What to assess.

Domain	Questions
Commitment	"Is there any part of you that has already decided to leave?"
History	How did you meet? What drew you? What was good in the early years?
Current pattern	"Walk me through last Tuesday's argument, minute by minute."
Attempted solutions	What have you tried? What happened?
Family of origin	Genogram of both families; how each family handled conflict, affection, money
Context	Work, money, migration, in-laws, children, faith, health
Strengths	What still works? What do you still enjoy?
Risk	Violence, coercive control, suicide, substance use, child safety

Ask about the good. Couples in distress systematically under-report positives; a careful strengths inventory is both assessment and intervention.

Week 3 Individual sessions, IPV screening and secrets

Always see each partner alone at least once. This is where safety is disclosed.

Screening questions (ask directly, in private).

- "Have you ever felt afraid of your partner?"
- "Has your partner ever hit, pushed, slapped, or thrown something at you?"

- "Has your partner ever forced or pressured you sexually?"
- "Does your partner control your money, your phone, or who you see?"
- "If you went home tonight and told him you had said all this, what would happen?"

If coercive control is present: stop conjoint work; safety-plan individually; refer to specialist services; document. Conjoint work can increase danger and can silence the victim in the room.

Secrets policy. State it in the first joint session: "If one of you tells me something individually that affects our work together, I will not keep it as a secret. I will help you decide how and when to tell your partner, and if you cannot, I will ask us all to talk about it."

Ambivalence. One partner may already be planning to leave. Ask: "Where are you on a scale from 'I am fully committed to working on this' to 'I have already decided'?" The answer changes the treatment.

Week 4 The negative cycle

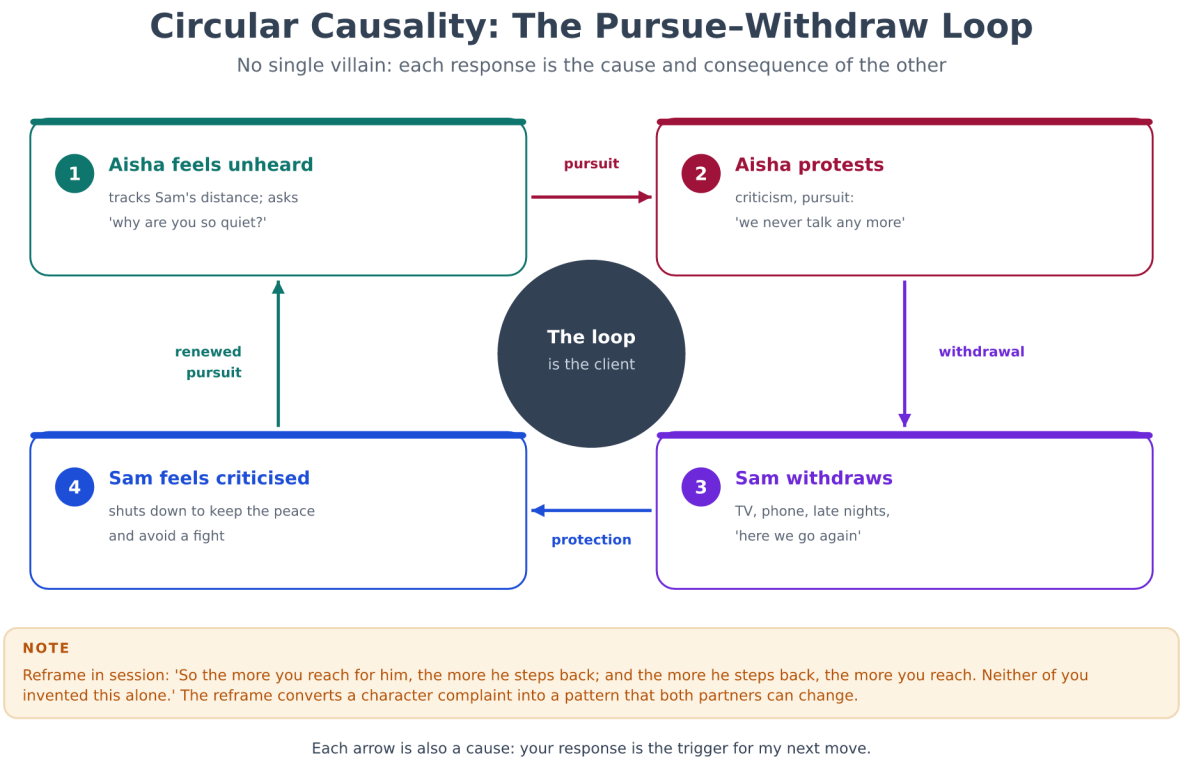


Figure 4.1 Circular causality: the pursue withdraw loop

The three common cycles.

1. **Pursue withdraw.** One escalates to get a response; the other shuts down to keep the peace. The pursuing increases, the withdrawal deepens.

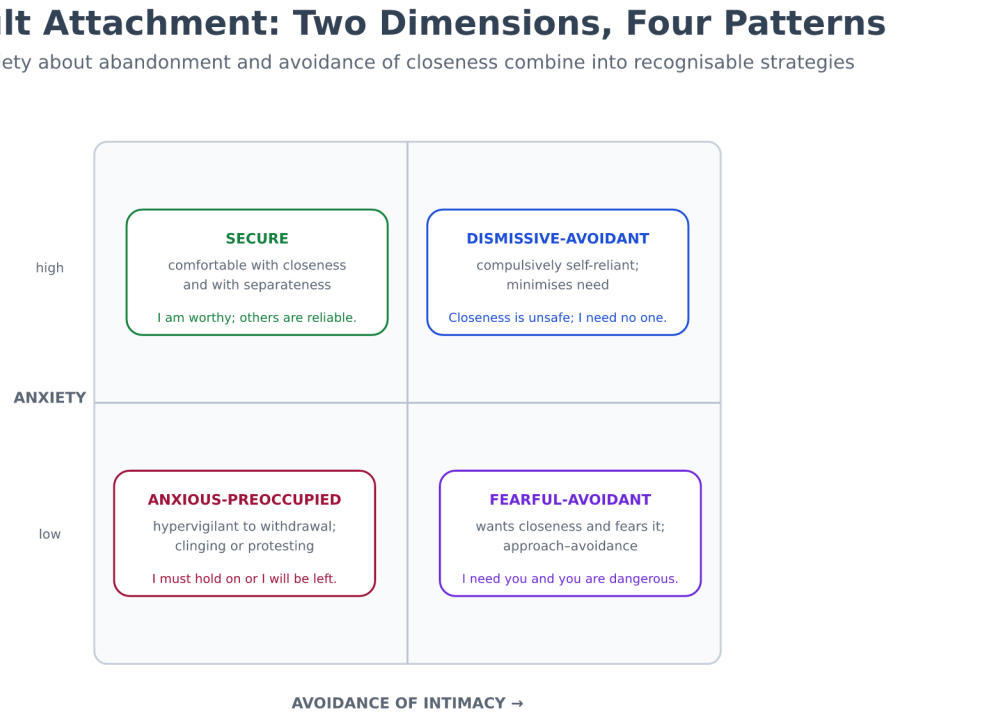
2. **Attack defend.** Both escalate; complaint is met with counter-complaint; the volume rises and the content becomes irrelevant.
3. **Mutual withdrawal.** Both avoid; the relationship becomes parallel and polite; loneliness without conflict often the hardest to treat because there is no presenting crisis.

Mapping the cycle in session. Ask each partner for the last three arguments and write the sequence on paper as a loop. Then read it back and ask: "Is this recognisable?" The moment of recognition often accompanied by a laugh or a sigh is the therapeutic turn.

The reframe. "The problem is not that you are angry, or that you are cold. The problem is the loop between you, and the loop has a life of its own."

> **PRACTICAL**
> With a role-played couple, map the cycle on a whiteboard in 15 minutes and get both partners to agree with the drawing. The agreement, not the drawing, is the skill.

Week 5 Attachment applied to couples



> **NOTE**
> Attachment is a pattern, not a personality type. The same person can be secure with a sibling and anxious with a partner, and patterns shift with context, therapy and a safe relationship. Assess the pattern, then ask what it protects.

Figure 5.1 Adult attachment: two dimensions and four patterns

Core idea. Adult romantic love is an attachment process. Partners monitor accessibility and responsiveness; when the bond feels threatened, protest behaviours appear (criticism, pursuit, withdrawal, numbing), and those protest behaviours are usually the opposite of the need they express.

Two dimensions. Anxiety (about abandonment) and avoidance (of intimacy). Their combination produces four patterns, each with a characteristic strategy and core belief (see figure).

Clinical use.

- Anxious partner: the protest is a request for reassurance. Help them ask for it directly rather than through criticism.
- Avoidant partner: withdrawal is a protection against overwhelm and shame. Help them stay present for longer and return rather than disappear.
- Both: the cycle is an attachment problem, not a character problem.

The attachment question to hold in your head. "What is this person asking for, and what are they doing instead?"

Week 6 EFT I: tracking and de-escalation

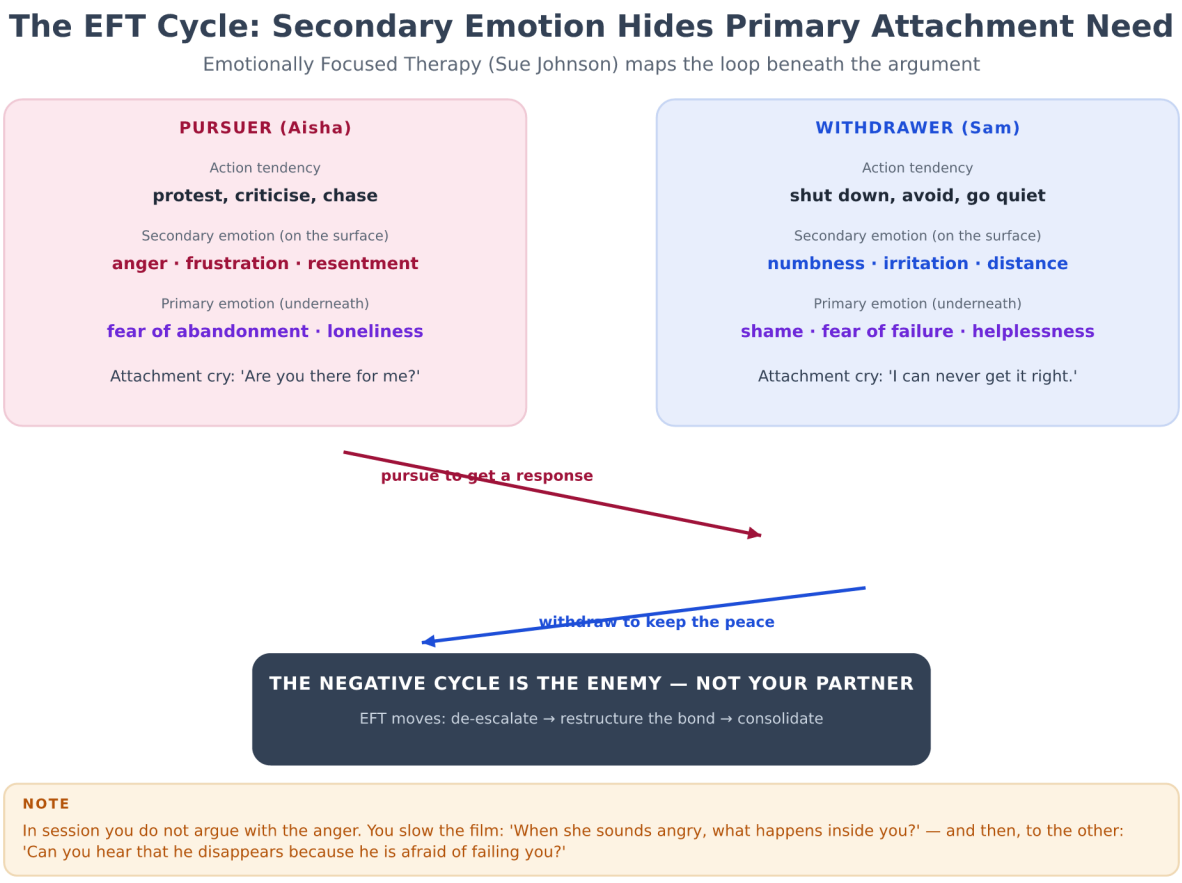


Figure 6.1 The EFT cycle: secondary emotion and primary attachment need

Stage 1 (de-escalation, sessions 1 8). The goal is that both partners can describe the cycle and see it as the enemy.

Four moves.

1. **Track the cycle** in specific incidents. Slow the film; ask for bodily experience.
2. **Access primary emotion.** "Under the anger is there something like fear?" Name it tentatively; let them correct you.
3. **Reframe the problem as the cycle.** "Neither of you is the problem. This dance is."
4. **Name the attachment cry.** "So when you say 'we never talk', what you mean is 'I miss you and I am afraid you have gone'."

A worked enactment (Week 6 lab).

> **THERAPIST (TO AISHA):**
> When you get loud, what happens inside your body? **Aisha:** My chest. Like I cannot breathe until he answers. **Therapist:** And if you did not get loud what would he see? **Aisha:** (pause) That I am frightened. **Therapist:** Can you tell him that? Look at him. "I get loud because " **Aisha:** I get loud because I think you have already left. **Therapist (to Sam):** Sam, stay with that. What is happening for you hearing it?

Hazard. Primary emotion must be reached, not demanded. If you push a client to feel something in front of you before safety, you will get a performance or a shutdown.

Week 7 Observed couple assessment

Forty minutes: contract, both accounts, cycle map, risk screen (in the individual sessions if scheduling permits), strengths, and feedback. Assessed on safety, structure, balance (did each partner get equal airtime?), and formulation quality.

Week 8 EFT II: restructuring

Stage 2 (sessions 9 20). Withdrawal re-engagement; pursuer softening; enactment of new responses.

Softening event the classic sequence.

1. The pursuer accesses the primary attachment fear and expresses it non-accusatorially.
2. The withdrawer hears it, stays present, and responds with care.
3. The pursuer risks reaching again from a position of vulnerability rather than protest.
4. Both experience a new pattern in the room.

Therapist tasks. Keep the withdrawer engaged ("stay with me, this is the hard part"); block the pursuer's escalation without silencing the need; and slow everything down.

Stage 3 (consolidation). Integrate the new pattern, problem-solve practical issues, and rehearse recovery from relapse: "When the cycle comes back and it will what will you do in the first hour?"

Week 9 Behavioural couple therapy

The behavioural view. Distress is maintained by (a) a low rate of positive exchange, (b) poor communication under conflict, and (c) attributions that make neutral behaviour hostile.

Core interventions.

1. **Behavioural exchange.** Each partner lists 15 20 small caring behaviours; select three each; do them daily without being asked. Not transactional bookkeeping an increase in the density of positive behaviour.
2. **Communication training.** Speaker listener with a floor card, then problemsolving.
3. **Attribution retraining.** "When he is late, what do you assume? What else could be true?"
4. **Acceptance work (IBCT).** Some differences will not change; the work is to soften the emotional reactivity to them through empathic joining and tolerance building.

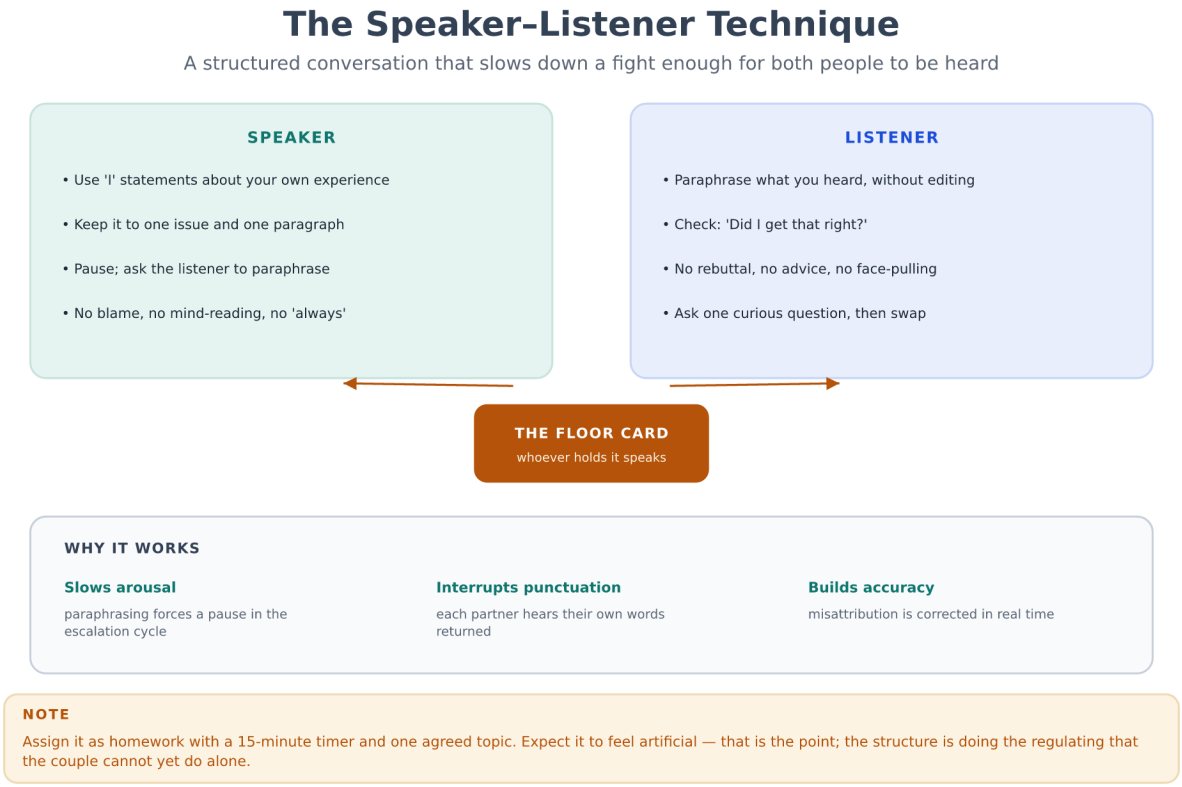
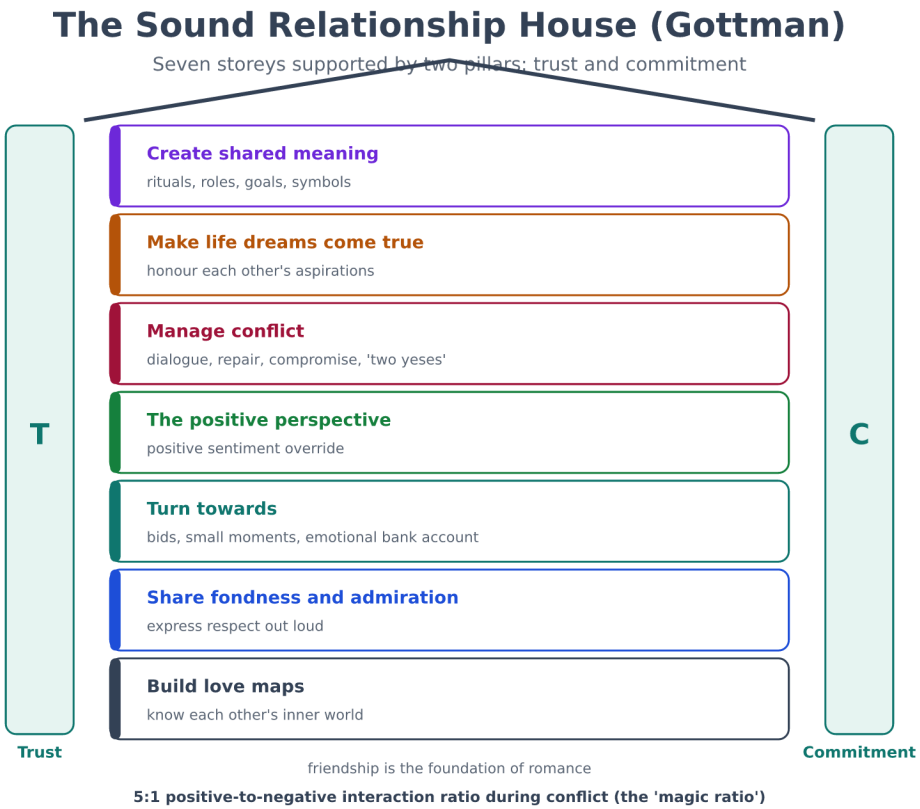


Figure 9.1 The speaker listener technique

Sequencing. Positives first (exchange), then communication, then problem-solving. Couples cannot solve problems they cannot discuss.

Week 10 The Gottman research programme



NOTE

Adaptation caution: Gottman's norms were derived largely from U.S. samples. Use the structure of the house as a clinical map; do not treat the 5:1 ratio as a universal law.

Figure 10.1 The Sound Relationship House

Findings worth using.

- **The four horsemen** criticism, contempt, defensiveness, stonewalling predict dissolution (Figure 14). Contempt is the strongest predictor.
- **The 5:1 ratio** of positive to negative interaction during conflict.
- **Bids for connection** small moments of "turning towards" build the emotional bank account.
- **Love maps** detailed knowledge of the partner's inner world.
- **Repair attempts** the capacity to de-escalate mid-conflict is more predictive than the absence of conflict.
- **Physiological flooding** above roughly 100 bpm, people cannot listen; a break is a skill, not an escape.

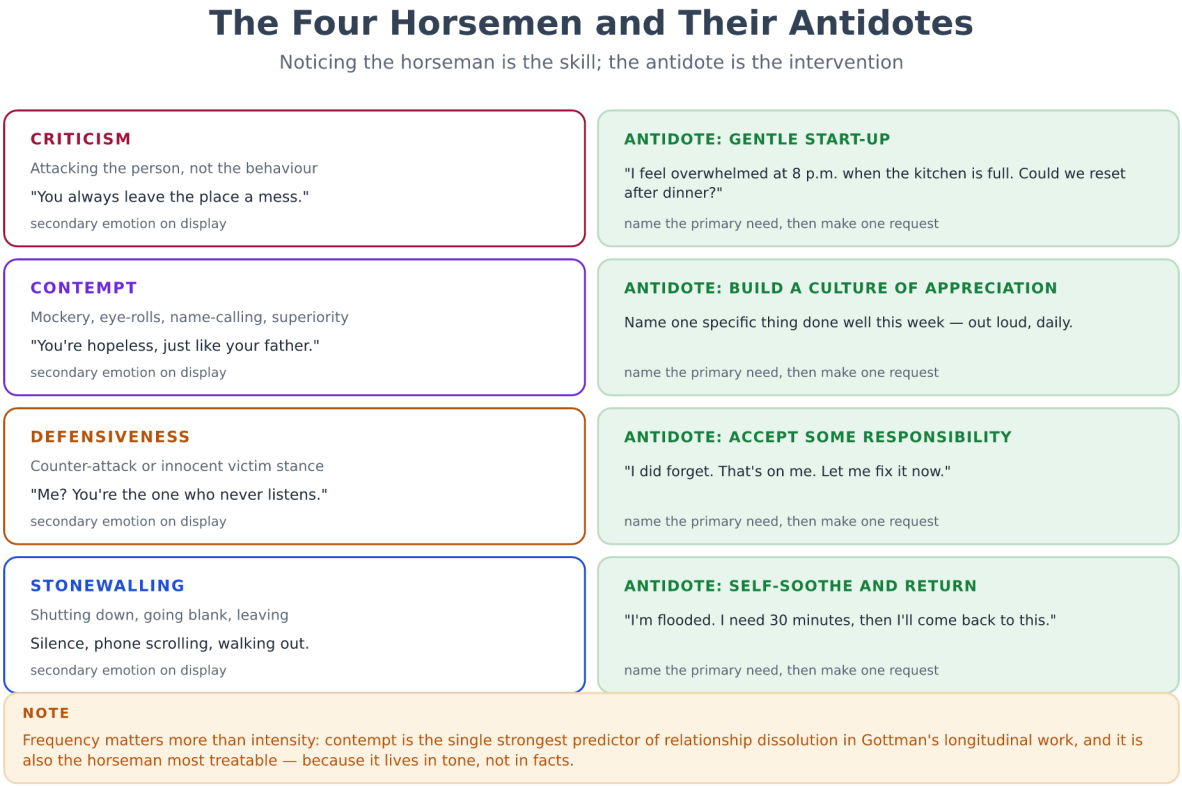


Figure 10.2 The four horsemen and their antidotes

Adaptation caution. The norms come largely from US samples; treat the structure as a clinical map, not as a universal law. A couple who argue loudly and repair well is not in trouble because of the volume.

Week 11 Infidelity and betrayal trauma

Two clinical events, often confused.

1. **The disclosure** an acute crisis requiring containment, safety and structure.
2. **The betrayal trauma** an injury to the betrayed partner's capacity to trust and to make sense of reality, with symptoms resembling PTSD (intrusion, hypervigilance, dissociation).

Phase 1: stabilise (weeks 1 4).

- Stop the affair (or the contact) this is usually a precondition, not a negotiation.
- Establish safety: no violence, no coercion, agreed boundaries on phone and whereabouts.
- Contain the disclosure: the betrayed partner has a right to the facts, not to every detail; agree what will be asked and answered.
- Address STI testing and any financial/legal exposure.
- Individual sessions to assess each partner's capacity to continue.

Phase 2: meaning (weeks 4 12).

- What did the affair do for the unfaithful partner? (Not "why did you do it?" but "what was it in service of?")
- What was missing that could have been spoken? (Careful: explanation is not justification.)
- What does the betrayed partner need in order to consider rebuilding transparency, remorse, time, consistency?

Phase 3: rebuilding or separating. Not every couple should stay together. Your task is to help them decide with full information, not to preserve the marriage at all costs.

> **CAUTION**
> Do not accept a couple for affair work if the unfaithful partner will not end the affair and will not disclose anything; you will be running a therapy that cannot progress and colluding in a lie.

Week 12 Sex and intimacy

Start with permission, then information, then specific suggestions (Annon's PLISSIT model).

Assessment. Desire, arousal, orgasm, pain; frequency and satisfaction for each partner; the last time it was good; what changed; medical factors (medication, illness, menopause, postpartum); the couple's beliefs; and the meaning of sex in the relationship.

Common presentations and the couple-level formulation.

Presentation	Individual factor	Relational factor
Low desire in one partner	Fatigue, medication, depression, pain	Pursuit and pressure; unresolved resentment
Erectile difficulty	Vascular, hormonal, anxiety	Performance focus; fear of judgement
Painful intercourse	Medical cause must be excluded	Fear, poor communication, coercion
Mismatched desire	Cycles differ normally	Negotiation failure; rejection sensitivity

Intervention basics. Psychoeducation (desire discrepancy is the most common sexual complaint, not a sign of incompatibility); sensate focus (graded, non-goal-oriented touch); scheduling; reducing performance pressure; addressing the relational context; and medical referral when indicated.

Cultural and religious framing. Sexual expectations are among the most culturally loaded topics you will discuss. Ask what each partner was taught, what they believe is permitted, and whether they can say no to each other. Consent within marriage is consent.

Week 13 Violence, control and contraindications

The distinction that matters (from MFT 704):

Situational couple violence	Coercive control
Mutual, escalating under stress	One-directional domination
No fear outside conflict	Chronic fear, surveillance, isolation
Couple therapy possible after assessment	Couple therapy contraindicated
Safety plan for both	Specialist advocacy for the victim

If a couple presents with violence. See each partner individually first. Ask about fear, injury, sexual coercion, threats, and escalation. Do not use conjoint sessions to "work through" violence; do not ask the victim to say in front of the perpetrator what he did, unless she chooses and it is safe.

REFLECT Return to your Week 1 reflection. Where in this material are you most likely to lose balance with the victim, with the perpetrator, with the unfaithful partner, with the one who wants to leave?

Week 14 Separation, divorce and stepfamilies

Discernment counselling. Some couples are not deciding whether to repair the relationship but whether to end it. A structured, time-limited process (about 4 6 sessions) helps a couple reach a clear decision rather than drifting. Both outcomes a renewed commitment or a respectful ending are successes.

When the decision is to separate.

- Help the couple tell the children together, with a simple, blame-free message.
- Refer for legal advice on custody, maintenance, property and any customary or religious processes.
- Address co-parenting: the relationship continues as a parenting partnership.
- Manage the grief: divorce is a loss, even when it is a relief.

Stepfamilies. The central error is treating the stepfamily as a nuclear family that arrived late. It is a different structure with different tasks: the couple bond must be built before stepparent authority is imposed; children need permission to love both households; and the "insider/outsider" split (biological parent and child versus stepparent) must be named.

Week 15 Culture, faith and family of origin

Questions to ask every couple.

- What did each of your families teach you about marriage, money, sex, and in-laws?
- Who has authority in your household, and who decided that?

- What does your faith require of each of you?
- Which of your families is closer, and what does that closeness cost the other?
- If your parents disagreed with a decision you made, what would happen?

Common cultural configurations. Bride price and its obligations; polygyny; co-residence with parents; gender-role expectations; religious courts and counselling; migration and split households; inter-ethnic and inter-faith marriages.

The rule. Do not treat the cultural pattern as the problem, and do not treat it as beyond discussion. Ask the couple what they want within it.

Week 16 Consolidation and ending

Write the couple a one-page **relapse prevention plan**: their cycle in their own words, three early warning signs, two repair moves that work for them, and the agreement about when to return. Review the gains explicitly, name the risk of recurrence without frightening them, and end on time.

Glossary

Attachment injury a betrayal or abandonment that damages trust and requires explicit repair. **Betrayal trauma** trauma-like symptoms following the discovery of a partner's deception. **Bids for connection** small attempts to gain a partner's attention or affection. **Coercive control** a pattern of domination and isolation, distinct from situational conflict. **Negative cycle** the recurrent pattern of interaction that organises couple distress. **Pursue withdraw** a cycle in which one partner escalates while the other distances. **Softening** in EFT, the pursuer's shift from protest to vulnerable expression. **Sentiment override** the tendency for the relationship's overall tone to colour the interpretation of specific events. **Sensate focus** graded, non-goal-oriented touch exercises used in sex therapy.

Self-assessment

Multiple choice

1. The first step in assessing a couple is: (a) communication training (b) assessment including individual sessions and IPV screening (c) homework (d) diagnosis

2. In EFT, criticism is usually understood as: (a) abuse (b) protest at perceived abandonment (c) personality (d) a communication deficit

3. Couple therapy is contraindicated when: (a) the couple argues (b) there is coercive control (c) there is low desire (d) one partner is depressed

4. The strongest predictor of relationship dissolution in Gottman's research is: (a) conflict frequency (b) contempt (c) low income (d) different interests

5. A 5:1 ratio refers to: (a) sessions per month (b) positive to negative interactions during conflict (c) partners to therapist (d) years of marriage

6. In affair work, the precondition for progress is usually: (a) forgiveness (b) ending the affair and establishing transparency (c) a legal separation (d) a new sexual contract

7. "Softening" in EFT refers to: (a) lowering the volume (b) the pursuer expressing primary attachment emotion (c) the therapist's tone (d) reducing homework

8. Physiological flooding is significant because: (a) it indicates anger (b) above a threshold, listening becomes physiologically difficult (c) it predicts divorce (d) it requires medication

9. A key task in stepfamilies is to: (a) impose stepparent authority quickly (b) build the couple bond before stepparent authority (c) treat all children equally in discipline (d) avoid the ex-partner

10. Discernment counselling helps couples to: (a) stay together (b) reach a clear decision about the relationship (c) divorce quickly (d) avoid decisions

Answer key

1 b 2 b 3 b 4 b 5 b 6 b 7 b 8 b 9 b 10 b

Short answer

1. Give the three-session structure of a couple assessment and the purpose of each session.
2. Write your confidentiality policy for secrets in couple therapy, in six sentences.
3. Describe the pursue withdraw cycle in attachment terms.
4. Name three contraindications to conjoint couple therapy.
5. What are the phases of infidelity work, and what happens in each?

Essay (choose one, 2,000 words)

1. Compare EFT and behavioural couple therapy for a highly distressed couple with a pursue withdraw cycle.
2. "Infidelity is a symptom and an injury." Discuss the treatment implications.
3. Critically evaluate the cultural assumptions embedded in the concept of "differentiation" in couple therapy.

Further reading

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Next: MFT 711 Child and Adolescent Therapy